

Regulatory, Compliance & Data Law Reference

The laws that govern Sachetana: the service mandate, and the statutes governing the data it would collect.

Prepared for **MAHE, discovery discussion (review panel)**

Date **14 September 2026**

Scope **2 mandates • 7 statutes/regulations**

I. The Service Mandate

Supreme Court of India: Sukdeb Saha v. State of Andhra Pradesh 25 July 2025

Directs the Government of India to frame a uniform mental health policy for all higher educational institutions, built on the UMMEED guidelines, the Manodarpan initiative, and the National Suicide Prevention Strategy. Establishes binding minimum requirements: one qualified counsellor, psychologist or social worker at any institution with 100 or more students; a mental health and wellbeing centre on campus; peer supporters at a ratio of 1:100 and faculty mentors at 1:500; a 24×7 helpline integrated with the national Tele-MANAS service; and staff training delivered twice a year by certified professionals.

University Grants Commission: Advisory on Tele-MANAS Integration 13 November 2025

Directs higher educational institutions to actively promote Tele-MANAS (14416), the national tele-mental-health programme, across campuses.

II. Data Protection & Privacy Law (India)

Digital Personal Data Protection Act, 2023, with DPDP Rules, 2025 phasing in to 2027

Governs all personal data processed digitally, including health data, with no separate “sensitive data” category. Anyone under 18 requires verifiable parental consent and cannot be profiled or shown targeted content, unless the processing is carried out by a clinical establishment or mental health institution, an exemption written into DPDP Rules, Rule 11 read with Schedule IV.

Information Technology (Reasonable Security Practices and Sensitive Personal Data or Information) Rules, 2011 “SPDI Rules”

Pre-DPDP baseline. Rule 3 explicitly classifies physical, physiological and mental health condition, and medical records and history, as sensitive personal data. Remains the operative definitional standard for “is mental health data sensitive” until the DPDP Rules' consent and security provisions fully commence.

Mental Healthcare Act, 2017 (Section 23)

Establishes a right to confidentiality for any person receiving mental healthcare, extending explicitly to information stored in electronic or digital format, in real or virtual space. Disclosure is permitted only to a nominated representative, or to prevent harm and only on the order of a High Court or the Supreme Court. No lower bar, institutional exception, or research carve-out exists.

Ayushman Bharat Digital Mission: Health Data Management Policy

Establishes the consent-manager framework behind Ayushman Bharat Health Accounts: explicit, purpose-specific consent, and a right to erasure via Health ID deletion. Linking into ABDM is optional, and adds a second consent layer on top of the DPDP Act and the Mental Healthcare Act rather than replacing either.

III. Global Comparators

Health Insurance Portability and Accountability Act (HIPAA) *United States*

Governs protected health information held by covered entities (healthcare providers, health plans, and their business associates) in clinical relationships. A direct-to-consumer wellness application can operate entirely outside HIPAA by never becoming a covered entity.

General Data Protection Regulation (GDPR), Article 9 *European Union*

Classifies health data as a special category of personal data, requiring explicit consent or another specified legal basis before it may be processed.

Family Educational Rights and Privacy Act (FERPA) *United States*

Governs student education records at US institutions. Campus counselling records fall under FERPA rather than HIPAA, per joint federal guidance.